LABORATORY REPORT

Redcliffe labs

Patient NAME	**: Mr sachin**		
DOB/Age/Gender	: 49 Y/Male	Report STATUS	: Final Report
Patient ID / UHID	: 19220864/RCL18875122	Barcode NO	: 35818131
Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: Oct 10, 2026, 08:36 AM	Report Date	: Oct 10, 2026, 10:55 AM.

NABL-M(EL)T-03144 MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

LoopHealth - Pharma Essential

Complete Blood Count (CBC)

Test Description	Value(s)	Unit(s)	Reference Range
RBC Parameters			
Hemoglobin *Spectrophotometry (Cyanide Free)*	**12.9 L***	g/dL	13.0 - 17.0
RBC Count *Electrical impedance*	4.6	10^6/µl	4.5 - 5.5
PCV *Calculated*	**39.2 L***	%	40 - 50
MCV *Calculated*	84.5	fl	83 - 101
MCH *Calculated*	27.7	pg	27 - 32
MCHC *Calculated*	32.8	g/dL	31.5 - 34.5
RDW (CV) * *Calculated*	**14.2 H***	%	11.6 - 14.0
RDW-SD * *Calculated*	43	fl	35.1 - 43.9
WBC Parameters			
TLC *Electrical impedance and microscopy*	5.7	10^3/µl	4 - 10
Differential Leucocyte Count			
Neutrophils *Laser based Flow-cytometry*	62	%	40-80
Lymphocytes *Laser based Flow-cytometry*	24	%	20-40
Monocytes *Laser based Flow-cytometry*	4	%	2-10
Eosinophils *Laser based Flow-cytometry*	**10 H***	%	1-6
Basophils *Laser based Flow-cytometry*	0	%	<2
Absolute Leukocyte Counts			
Neutrophils. * *Calculated*	3.53	10^3/µl	2 - 7
Lymphocytes. * *Calculated*	1.37	10^3/µl	1 - 3
Monocytes. * *Calculated*	0.23	10^3/µl	0.2 - 1.0

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Test Description	Value(s)	Unit(s)	Reference Range
Eosinophils. * *Calculated*	**0.57 H***	10^3/µl	0.02 - 0.5
Basophils. * *Calculated*	**0**	10^3/µl	0.02 - 0.5
Platelet Parameters			
Platelet Count *Electrical impedance*	162	10^3/µl	150 - 410
Mean Platelet Volume (MPV) * *Calculated*	10.3	fL	9.3 - 12.1
PCT * *Calculated*	0.2	%	0.17 - 0.32
PDW * *Calculated*	16.1	fL	8.3 - 25.0
P-LCR * *Calculated*	29.1	%	18 - 50
P-LCC * *Calculated*	47	10^9/L	44 - 140
Mentzer Index * *Calculated*	18.37	%	> 13

Interpretation:
CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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Sample Collected	: Oct 10, 2026, 08:36 AM	Report Date	: Oct 10, 2026, 11:46 AM.

Erythrocyte Sedimentation Rate (ESR)

Test Description	Value(s)	Unit(s)	Reference Range
ESR - Erythrocyte Sedimentation Rate *MODIFIED WESTERGREN*	**22 H***	mm/hr	0 - 10

Interpretation:

ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis ,systemic vasculitis, anemia, multiple myeloma , etc. Low levels are typically seen in congestive heart failure, polycythemia ,sickle cell anemia, hypo fibrinogenemia , etc.

Reference- Dacie and lewis practical hematology

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Patient NAME	**: Mr sachin**		
DOB/Age/Gender	: 49 Y/Male	Report STATUS	: Final Report
Patient ID / UHID	: 19220864/RCL18875122	Barcode NO	: 35817102
Referred BY	: Self	Sample Type	: Serum
Sample Collected	: Oct 10, 2026, 08:36 AM	Report Date	: Oct 10, 2026, 11:49 AM.

NABL-M(EL)T-03144 MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Liver Function Test (LFT)

Test Description	Value(s)	Unit(s)	Reference Range
Bilirubin Total *DIAZO*	0.4	mg/dL	0.1 - 1.2
Bilirubin Direct *Diazo Reaction*	0.2	mg/dL	0 - 0.5
Bilirubin Indirect * *Calculated*	0.2	mg/dL	0.1 - 1.0
SGOT/AST *IFCC without PDP*	21.1	U/L	5 - 35
SGPT/ALT *IFCC without PDP*	31	U/L	5 - 45
SGOT/SGPT Ratio * *Calculated*	0.68	Ratio	<1.00
Alkaline Phosphatase *AMP buffer*	121	U/L	53 - 128
Total Protein *Biuret*	7	g/dL	6.4 - 8.3
Albumin *Colorimetric BCG*	3.8	g/dL	3.5 - 5.2
Globulin * *Calculated*	3.2	g/dL	2.3 - 3.5
Albumin :Globulin Ratio * *Calculated*	**1.19 L***	-	1.3 - 2.1
Gamma Glutamyl Transferase (GGT) * *Glupa-C*	21	U/L	12 - 64

Interpretation:

The liver filters blood, metabolizes nutrients, detoxifies harmful substances, and produces blood clotting proteins. Liver cells contain enzymes that facilitate these functions. When cells are damaged, enzymes leak into the blood, detectable through blood tests.

Key enzymes tested:

1. AST (SGOT): may indicate tissue injury / damage in muscles or liver.

2. ALT (SGPT): Primarily in the liver. Elevated ALT and AST suggest liver damage.

3. Alkaline Phosphatase & GGT: Linked to bile production and flow. Elevated levels may indicate bile flow issues related to the liver, gallbladder, or bile ducts.

Blood proteins, **albumin and globulin**, are essential for growth, development, and health.

1. Low protein: May indicate bleeding, liver disorders, malnutrition, or agammaglobulinemia.

2. High protein (Hyperproteinemia): Often due to dehydration or increased protein production.

3. Low albumin: Caused by poor diet, kidney, or liver disease.

4. High albumin: Usually due to severe dehydration.

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NABL-M(EL)T-03144 MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Kidney Function Test (KFT)

Test Description	Value(s)	Unit(s)	Reference Range
Blood Urea *Urease-GLDH*	**18.2 L***	mg/dL	19 - 45
Bun * *Calculated*	8.5	mg/dL	6 - 20
Creatinine *Photometric*	1.1	mg/dL	0.7 - 1.3 mg/dL
eGFR (CKD-EPI) *	82.28	ml/min/1.73 sq m	Normal Or High: >= 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15
Bun/Creatinine Ratio * *Calculated*	**7.73 L***	Ratio	12 - 20
Urea / Creatinine Ratio * *Calculated*	**16.55 L***	Ratio	25.68- 42.8
Uric Acid *Uricase*	5.4	mg/dL	3.5 - 7.2 mg/dL
Calcium Serum *Arsenazo III*	9	mg/dL	8.4 - 10.2
Phosphorus *UV Molybdate*	3.5	mg/dL	2.5 - 4.5
Sodium *DIRECT ISE*	**131.9 L***	mmol/L	136 - 145
Potassium *DIRECT ISE*	4.1	mmol/L	3.5 - 5.1
Chloride *ISE*	97.6	mmol/L	95 - 107

Interpretation:

Kidney FUNCTION tests IS a collective term FOR a variety of individual tests AND procedures that can be done TO evaluate how well the kidneys are functioning. Many conditions can affect the ability of the kidneys TO carry OUT their vital functions. SOME lead TO a rapid (acute) decline IN kidney FUNCTION others lead TO a gradual (chronic) decline IN function. BOTH result IN a buildup of toxic waste substances done ON urine samples, AS well AS ON blood samples. A number of symptoms may indicate a problem WITH your kidneys. These include : high blood pressure, blood IN urine, frequent urges TO urinate, difficulty beginning urination, painful urination, swelling IN the hands AND feet due TO a buildup of fluids IN the body. A single symptom may NOT mean something serious. However, WHEN occurring simultaneously, these symptoms suggest that your kidneys are NOT working properly. Kidney FUNCTION tests can HELP determine the reason. Ionized calcium this test IF you have signs of kidney OR parathyroid disease. The test may also be done TO monitor progress AND treatment of these diseases.**"eGFR test IS applicable FOR patients aged 18 years OR more."**

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NABL-M(EL)T-03144

MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Lipid Profile

Test Description	Value(s)	Unit(s)	Reference Range
Total Cholesterol *CHOD - PAP*	**309 H***	mg/dL	<200
Triglycerides *GPO-Trinder*	**183.1 H***	mg/dL	<150
HDL Cholesterol *PEGME*	**23.8 L***	mg/dL	>40
Non HDL Cholesterol * *Calculated*	**285.2 H***	mg/dL	<130
LDL Cholesterol *Calculated*	**248.58 H***	mg/dL	<100
V.L.D.L Cholesterol * *Calculated*	**36.62 H***	mg/dL	< 30
Chol/HDL Ratio * *Calculated*	**12.98 H***	Ratio	0.0 - 5.0
HDL/ LDL Ratio * *Calculated*	**0.1 L***	Ratio	0.5 - 3.0
LDL/HDL Ratio * *Calculated*	**10.44 H***	Ratio	0.0- 3.0

Interpretation:

Lipid level assessments must be made following 10 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays. Intraindividual (within-person) variation in triglyceride (TG) levels is high, often showing a 12.9% to 40.8% variation in healthy individuals within 1 year,. This variability is driven by a combination of biological, lifestyle, and physiological factors that fluctuate rather than remaining constant.

Causes of variation include

- Diet: Levels spike 5–10 times after meals.Unhealthy high saturated fat diets like non veg diet sweetened beverages , fried or processed foods , and alcohol intake can significantly raise triglyceride values.
- Lifestyle: Obesity , sedentary lifestyle, and smoking can raise levels.
- Biology: Pregnancy (estrogen), aging, and conditions like Diabetes or Metabolic Syndrome cause major shifts.
- Drugs like : Beta-blockers, steroids, tamoxifen, thiazides, and oral contraceptives can raise triglyceride levels

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High

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NABL-M(EL)T-03144

Test Description	Value(s)	Unit(s)	Reference Range

<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >/= 50 mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age >/=45 years in Males & >/= 55 years in Females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

*** After an adequate non-pharmacological intervention for at least 3 months.**

References :

- Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology,2022,20,134-155.
- Nordestgaard, B. A Test in Context: Lipid Profile, Fasting Versus Nonfasting. JACC. 2017 Sep, 70 (13) 1637–1646.
- P.N.M. Demacker, R.W.B. Schade, R.T.P. Jansen, A. Van 't Laar, Intra-individual variation of serum cholesterol, triglycerides and high density lipoprotein cholesterol in normal humans, Atherosclerosis,
- Parhofer KG, Laufs U: The diagnosis and treatment of hypertriglyceridemia. Dtsch Arztebl Int 2019; 116: 825–32. DOI: 10.3238/arztebl.2019.0825

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Referred BY	: Self	Sample Type	: Serum
Sample Collected	: Oct 10, 2026, 08:36 AM	Report Date	: Oct 10, 2026, 11:53 AM.

Thyroid Profile Total

Test Description	Value(s)	Unit(s)	Reference Range
Triiodothyronine (T3) *ECLIA*	95.8	ng/dL	35 - 193
Total Thyroxine (T4) *ECLIA*	7	µg/dL	4.87 - 11.2
Thyroid Stimulating Hormone (Ultrasensitive) *ECLIA*	2.6	mIU/L	0.35 - 4.94

Interpretation:

Pregnancy	Refrence Range TSH
1st Trimester	0.3 -4.5
2nd Trimester	0.5 -4.6
3rd Trimester	0.8 - 5.2

Note: TSH levels are subject to circadian variation, reaching peak levels between 2-4 am. and at a minimum between 6-10 pm. The variation is of 50 %, hence time of the day has influence on the measured serum TSH concentrations. Apart from nocturnal circadian surge other biological factors responsible for TSH variation include pulsatile secretion, along with seasonal changes and lifestyle factors such as smoking, BMI, and diet. Other contributors include aging, non-thyroidal illnesses, medication interference, and rare analytical issues like macro-TSH

Clinical Use:

1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

Increased TSH: Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance

Decreased TSH: Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency

Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

1. Primary Hypothyroidism: High TSH levels.

2. Secondary/Tertiary Hypothyroidism: Low TSH levels.

3. Euthyroid Sick Syndrome: Abnormal thyroid test results due to non-thyroidal illnesses (NTI).

TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

TSH	T4	T3	Interpretation
High	Normal	Normal	Mild (subclinical) hypothyroidism
High	Low	Low Or Nomral	Hypothyroidism
Low	Normal	Normal	Mild (subclinical) hyperthyroidism
Low	High Or Normal	High Or Normal	Hyperthyroidism
Low	Low Or Normal	Low Or Normal	Nonthyroidal illness; pituitary (secondary) hypothyroidism
Normal	High	High	Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function)

References

- Van der Spoel E, Roelfsema F and van Heemst D (2021) Within-Person Variation in Serum Thyrotropin Concentrations: Main Sources, Potential Underlying Biological Mechanisms, and Clinical Implications. Front. Endocrinol. 12:619568. doi: 10.3389/fendo.2021.619568

*** End Of Report ***

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TERMS AND CONDITIONS

1. Laboratory reports are generated solely for informational and interpretational purposes for qualified medical practitioners and must be read in conjunction with the patient's clinical history, examination findings, and other diagnostic parameters. The laboratory does not provide medical advice, diagnosis, or treatment recommendations.
2. All the test results are based strictly on the specimen/sample and information received under the details provided at the time of registration. The patient/customer or referring entity is solely responsible for the accuracy and completeness of details furnished before sample submission. The laboratory shall not be liable for errors, misidentification, or incorrect results arising due to inaccurate, incomplete, or misleading information provided at the time of registration.
3. The patient/customers must disclose all relevant conditions including but not limited to fasting status, medications, dietary intake, alcohol consumption, lifestyle factors, ongoing treatments, and medical history that may affect test outcomes. The laboratory shall not be responsible for any deviation in results due to non-disclosure or incorrect disclosure of such information.
4. Test results are dependent on the quality, integrity, and timing of the specimen received. The laboratory reserves the right to reject or request repeat samples in cases of inadequate, hemolyzed, contaminated, or otherwise compromised samples. No liability shall arise for inability to process or delay due to such reasons.
5. Test results are subject to inherent biological variation and analytical variation depending on the nature of the test, methodology, instrumentation, and pre-analytical factors. While the laboratory follows established quality control and assurance standards, minor variations in results may occur and should be interpreted in conjunction with clinical findings, patient history, and other diagnostic information. The laboratory does not guarantee absolute accuracy, precision, or reproducibility of results across different testing instances or laboratories, and no liability shall arise on account of such inherent variations.
6. All turnaround times are indicative, not guaranteed. Delays may occur due to test complexity, sample condition, referral testing, or quality control procedures, logistics, or other operational constraints.
7. Reports may be released via digital or physical modes such as Email, SMS, WhatsApp, Mobile App, Web Portal, or Printed Copy, subject to completion of testing, quality validation, and payment clearance. Accessing the report through any digital medium shall constitute acceptance of electronic records.
8. All patient information and reports are treated as confidential and may be shared only with patient/customer, authorized healthcare professionals, regulatory bodies, or as required by law.
9. Laboratory reports are not valid for medico-legal purposes unless specifically requested, processed, and certified as such in accordance with applicable laws.
10. To the maximum extent permitted under applicable law, the laboratory shall not be liable for any direct, indirect, incidental, consequential, or special damages arising out of or in connection with the use of, reliance upon, or interpretation of the laboratory report. The laboratory shall have no liability for any clinical decisions, diagnosis, or treatment undertaken on the basis of the report. In all circumstances, the liability of the laboratory, if any, shall be strictly limited to the amount actually paid for the specific test(s) giving rise to such claim.
11. In case of critical, alarming, or unexpected test results, you are advised to contact the laboratory immediately for further guidance and thereafter consult a qualified medical practitioner without delay. Test results should always be interpreted in conjunction with the patient's clinical history and clinical findings.
12. Accessing reports through digital platforms constitutes acceptance of electronic delivery and record storage.
13. The laboratory shall not be liable for any delays in performance or failure to perform its obligations arising out or in connection with events beyond reasonable control, including but not limited to natural disasters, pandemics, acts of God, strikes, labor disputes, transportation disruptions, regulatory or governmental actions, technical failures, or any other unforeseen circumstances.
14. All disputes arising from laboratory services shall be subject to the jurisdiction of courts located in Delhi.